CIT 140 · Week 4 · Cedar Junction

Validated Requirements Set

Jose Romero · Revision 8977

R-01 · functional

Requirement: The system must distinguish held from ready because one label is hiding two different conditions, and the intake queue displayed the appointment as held.

Source: W04-S01 · Dana Ruiz

Rationale: It matters because in scheduling, held is used for a slot awaiting final confirmation. Whereas intake uses held for a visitor which has arrived but cannot proceed forward. Held has stopped the intake coordinator because scheduling and intake use it for different purposes.

Acceptance check: Given that one label is hiding two conditions, it would need to be checked by both scheduling and intake. It is important to note that Dana can confirm the intake workflow. However, she is not able to approve privacy rules or clinical capacity.

Priority: Must · **Validation:** Confirmed by Dana Ruiz

Conflict or decision: At intake, held means that a visitor has arrived but they cannot move forward. The intake queue displayed the appointment as held, it did not state ready.

R-02 · user

Requirement: When a status exception occurs the staff must be able to resolve the issue at hand. The staff need to see which condition applies, who changed it, and what the next approved step is.

Source: W04-S01 · Dana Ruiz

Rationale: It is important because as of now, with the issue at hand the visitor is left to wait while they call around. Even though the visitor showed a confirmation for 9:30 am.

Acceptance check: Given the status exception, the staff would be able to check but they need to see which condition applies, who changed it, and the next approved step.

Priority: Must · **Validation:** Confirmed by Dana Ruiz

Conflict or decision: At intake, held means that a visitor has arrived but they cannot move forward. The intake queue displayed the appointment as held, it did not state ready.

R-03 · business

Requirement: Prior to a same-day slot being offered, a room and qualified clinician need to be available.

Source: W04-S02 · Dr. Lena Okafor

Rationale: This matters because even though there may be an open time on the calendar, that does not prove that either one is actually ready. It is not the same as clinical readiness. Clinical operations needs the current readiness state prior to intake promising a start time.

Acceptance check: This can be checked when Lena, the clinical operations lead, validates the capacity and readiness rules.

Priority: Must · **Validation:** Confirmed by Dr. Lena Okafor

Conflict or decision: Before readiness is confirmed, a room and a qualified clinician need to be available. This needs to be true because a calendar opening does not prove either one is actually ready.

R-04 · functional

Requirement: When readiness changes those that are affected by it, the staff and visitor, need to get an updated expectation.

Source: W04-S02 · Dr. Lena Okafor

Rationale: This is important because clinical details are not needed, they only need the information that is required to act.

Acceptance check: For one to be able to check it, the technical implementation needs to be decided upon.

Priority: Must · **Validation:** Confirmed by Dr. Lena Okafor

Conflict or decision: Before readiness is confirmed, a room and a qualified clinician need to be available. This needs to be true because a calendar opening does not prove either one is actually ready.

R-05 · user

Requirement: Approved communication and support needs must remain available to the authorized staff through confirmation, rescheduling, and check-in.

Source: W04-S03 · Priya Shah

Rationale: This information matters because although the visitor's communication preference was visible to the access team, it was not in the intake queue. Therefore, even after arriving the visitor needed to once again explain the same need.

Acceptance check: For one to check the communication and support needs, the permissions for every field need to be approved. A support request must follow the appointment.

Priority: Must · **Validation:** Confirmed by Priya Shah

Conflict or decision: Authorized staff know how support is arranged because language, communication, and approved support needs have to remain available to the authorized staff through confirmation, rescheduling, and check-in.

R-06 · constraint

Requirement: The required alternative to the visitor portal is an equivalent phone or staff-assisted route.

Source: W04-S03 · Priya Shah

Rationale: This required alternative matters because the access to the service should not be dependent and constrained to just one digital channel, especially if a visitor cannot access the portal.

Acceptance check: This able to be checked when Priya, the patient access coordinator, validates the access and communication needs.

Priority: Must · **Validation:** Confirmed by Priya Shah

Conflict or decision: Authorized staff know how support is arranged because language, communication, and approved support needs have to remain available to the authorized staff through confirmation, rescheduling, and check-in.

R-07 · nonfunctional

Requirement: The shared displays must only show the minimum information that staff need for the task.

Source: W04-S04 · Jordan Kim

Rationale: The information on the shared displays matters because the shared displays should not expose details that the staff do not need. The detailed information needs to follow staff roles. Still, the staff also need sufficient information to act.

Acceptance check: Someone can check it when Jordan, the Health IT and Privacy Analyst, validates access, audit, and availability constraints.

Priority: Must · **Validation:** Confirmed by Jordan Kim

Conflict or decision: For the privacy, the shared displays must only show the minimum information that staff need for the task. The detailed access needs to follow staff roles. For the audit boundary, the change history needs to keep who made the change, the previous value, the new value, and the time. The history needs to be reliable and accessible during an exception review.

R-08 · functional

Requirement: The status-change history must retain the one who made the change, the prior value, the new value, and the time.

Source: W04-S04 · Jordan Kim

Rationale: This status-change history is important because the history needs to remain available during an exception review. The history needs to be reliable when a status changes.

Acceptance check: Given what the status-change history must retain, one can check it through validating clinical readiness, seeing as the status can change.

Priority: Must · **Validation:** Confirmed by Jordan Kim

Conflict or decision: For the privacy, the shared displays must only show the minimum information that staff need for the task. The detailed access needs to follow staff roles. For the audit boundary, the change history needs to keep who made the change, the previous value, the new value, and the time. The history needs to be reliable and accessible during an exception review.

Professional Guidance Assessment

IIBA: IIBA is a professional association that has standards and many resources which describe business analysis work. The way in which they shape the practice of business analysis helps to achieve greater outcomes.

ISO/IEC/IEEE 29148: ISO, IEC, and IEEE contribute toward developing internationally agreed upon standards, utilizing information such as publication and technology standards to envision a sustainable future, and they create the standards for electronic technologies. Together they publish ISO/IEC/IEEE 29148 which guides requirements quality and structure.

PMI: PMI focuses on project delivery, while Week 4 focuses on requirements and more so what the system is needed to do. Week 4 leans toward the functionality aspect.

Application: This guidance improved the requirements due to the way in which they helped clear up information. It improved requirement 3 for instance because it helped to pinpoint what was needed prior to a same-day slot being offered, along with determining the importance. It also helped to clarify the logic.

Prepared in Command Nexus. Canvas remains the official submission point.